

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: JBPeriod: June 23 to July 23, 2024Report Date: July 24, 2024Coverage: Bed: 609/744h (81.9%)
Chair: 502/744h (67.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours

Elevated Breathing 12 AM to 4 AM		Low HR 12 AM to 4 AM	
Vital Stat	Avg	Min	Max
Heart Rate	57 bpm	40 bpm	86 bpm
Breathing	14 brpm	9 brpm	30 brpm

Last 24 hours: three low heart rate episodes overnight.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
12 AM to 4 AM	7h	3	Elevated Breathing	14	9	30	Check SpO2 and lung sounds; assess for fluid overload or early infection
12 AM to 4 AM	7h	3	Low Heart Rate (brief)	42	40	69	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

Patient clinical status over 31 days from Jun 23 to Jul 23

HR Breathing

Elevated HR Jun 23 to Jul 12 (20d)	Elevated Breathing Jun 23 to Jun 29 (7d)	Elevated Breathing Jul 15 to Jul 22 (8d)	High HR Jul 08 to Jul 12 (5d)	Low HR Jun 30 to Jul 04 (5d)	Low HR Jul 13 to Jul 23 (11d)
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115 episodic events Detected, spanning 7 days total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Low Heart Rate, Very High Heart Rate. **Priority grade: Medium**

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
115	183h	Elevated Heart Rate	4	100%

Major Findings: Sustained elevated HR (avg 98, peak 131)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 08	12 AM to 1 AM	7h	6	High Heart Rate	104	45	127	Check temp, hydration, pain; review for arrhythmia or infection
Jun 30	7 AM to 8 AM	7h	7	Low Heart Rate	44	40	60	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jul 13	10 PM to 11 PM	61h	26	Low Heart Rate	43	40	74	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 23	12 AM to 1 AM	77h	53	Elevated Heart Rate	98	46	131	Check temp, hydration, pain; review for arrhythmia or infection
Jun 23	7 AM to 8 AM	10h	9	Elevated Breathing	28	13	48	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 15	7 PM to 8 PM	21h	14	Elevated Breathing	26	9	36	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 29	2 AM to 3 AM	6h	5	Very High Heart Rate (brief)	112	83	130	Check temp, hydration, pain; review for arrhythmia or infection

+ 1 additional condition(s) across High Breathing (brief); details in monitoring data.

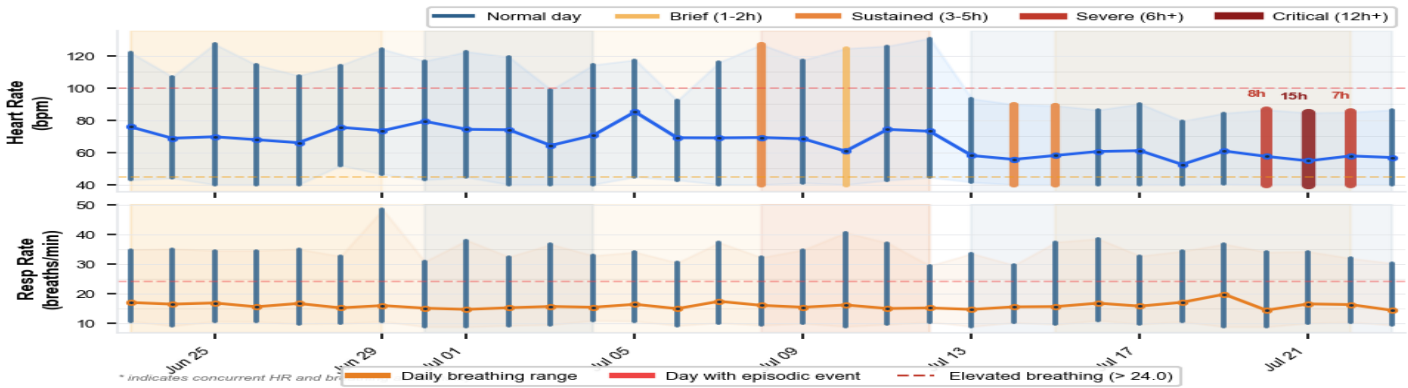
The P5 to P95 heart rate spread was 52 bpm (44 to 96), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 98 bpm, peak 131 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 26 brpm, peak 36 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 33 brpm, peak 37 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- High Heart Rate (avg 104 bpm, peak 127 bpm): Check temp, hydration, pain; review for arrhythmia or infection

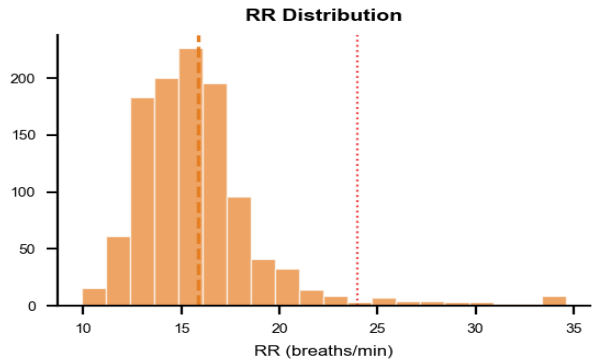
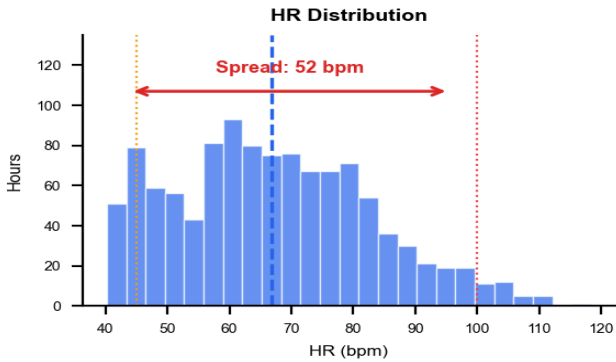
Clinical Pattern Observations:

- 1. **Sustained finding:** 8h continuous Low Heart Rate beginning Jul 20.
- 2. **High variability:** HR spread 52 bpm (43 to 95).
- 3. **Clustered pattern:** Episodic events on 7 of 31 days (22%).

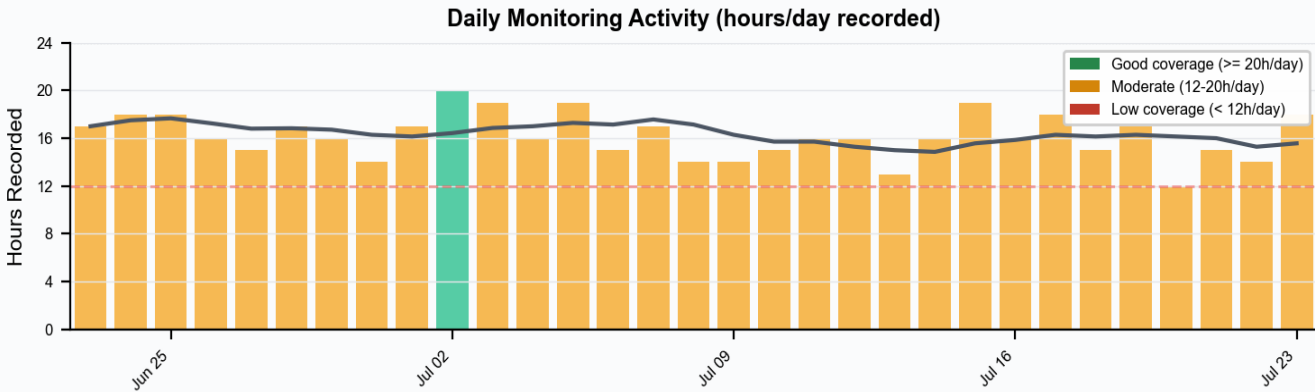


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.